TEST REPORT

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ZRT LABORATORY

 # 2026 05 21 404 S

Ordering Provider:
Rupa Health
Kirk Roberts

Reference Code
ord_j0vJy41

Samples Received
05/21/2026

Report Date
05/22/2026

Samples Collected
Saliva - 05/18/26 07:17
Saliva - 05/18/26 14:10
Saliva - 05/18/26 20:11
Saliva - 05/18/26 22:19

Patient Name: Rebecca DaSilva
Patient Phone Number: 954 204 6756

Gender Female	**Last Menses** 04/27/2026	**Height** 5 ft 11 in	**Waist** Unspecified
DOB 12/29/1995 (30 yrs)	**Menses Status** Pre-Menopausal	**Weight** Unspecified	

TEST NAME	RESULTS | 05/18/26	RANGE
Salivary Steroids & Other Analytes (LC-MS/ECLIA)		
Cortisol	4.8	2.88-7.12 ng/mL (morning)
Cortisol	0.6 L	1.11-2.74 ng/mL (noon)
Cortisol	<0.2 L	0.61-1.33 ng/mL (evening)
Cortisol	0.3	0.25-0.64 ng/mL (night)

<dl = Less than the detectable limit of the lab. ***N/A*** *= Not applicable; 1 or more values used in this calculation is less than the detectable limit.* ***H*** *= High.* ***L*** *= Low.*

Therapies

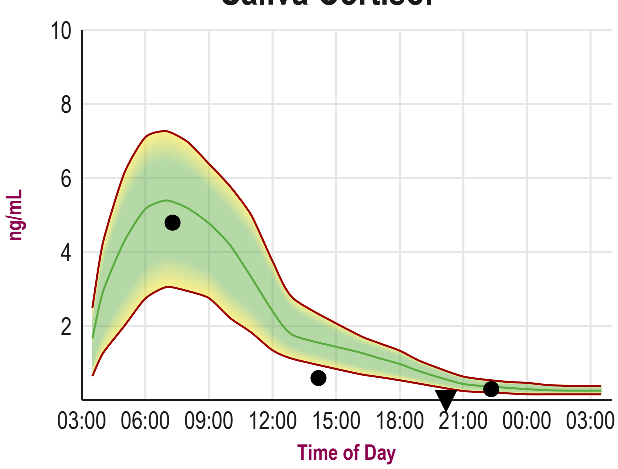
125mcg sublingual (SL) Vitamin D (unknown type) (OTC) (1 Days Last Used)

Graphs

Disclaimer: Graphs below represent averages for healthy individuals not using hormones. Supplementation ranges may be higher. Please see supplementation ranges and lab comments if results are higher or lower than expected.

Average ▼▲ Off Graph

Saliva Cortisol



The above results and comments are for informational purposes only and are not to be construed as medical advice. Please consult your healthcare practitioner for diagnosis and treatment.

David T. Zava, Ph.D.
Laboratory Director

Alison McAllister, ND.
(Ordering Provider unless otherwise specified on page 1)

Disclaimer: Supplement type and dosage are for informational purposes only and are not recommendations for treatment. For a complete listing of reference ranges, go to www.zrtlab.com/reference-ranges.

TEST NAME	WOMEN
Salivary Steroids & Other Analytes (LC-MS/ECLIA)	
Cortisol	2.88-7.12 ng/mL (morning); 1.11-2.74 ng/mL (noon); 0.61-1.33 ng/mL (evening); 0.25-0.64 ng/mL (night)

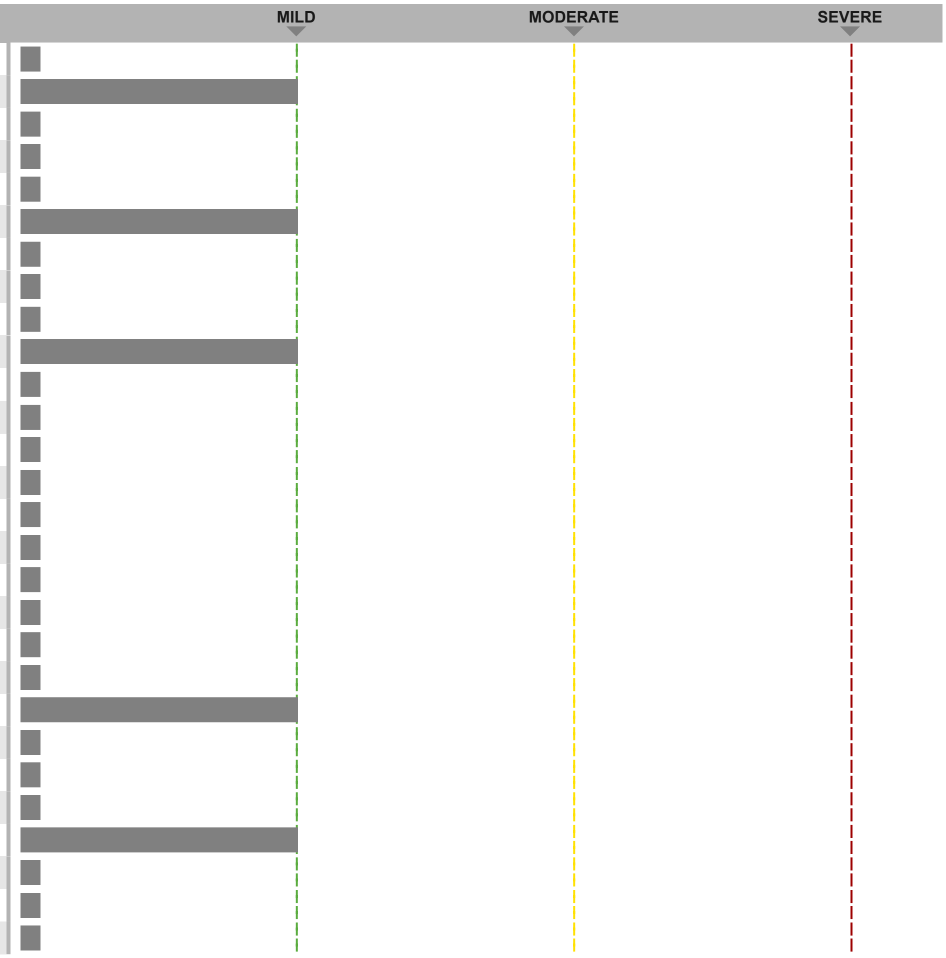
Disclaimer: Symptom Categories below show percent of symptoms self-reported by the patient compared to total available symptoms for each category. For detailed information on category breakdowns, go to www.zrtlab.com/patient-symptoms.

SYMPTOM CATEGORIES	RESULTS | 05/18/26
Estrogen / Progesterone Deficiency	15%
Estrogen Dominance / Progesterone Deficiency	10%
Low Androgens (DHEA/Testosterone)	7%
High Androgens (DHEA/Testosterone)	43%
Low Cortisol	5%
High Cortisol	5%
Hypometabolism	9%
Metabolic Syndrome	4%

SYMPTOM CHECKLIST	MILD / MODERATE / SEVERE
Aches and Pains	
Acne	
ADD/ADHD	
Addictive Behaviors	
Allergies	
Anxious	
Autism Spectrum Disorder	
Bleeding Changes	
Blood Pressure High	
Blood Pressure Low	
Blood Sugar Low	
Body Temperature Cold	
Bone Loss	BLANK
Breast Cancer	
Breasts - Fibrocystic	
Breasts - Tender	
Chemical Sensitivity	
Cholesterol High	
Constipation	
Depressed	
Developmental Delays	
Eating Disorders	
Fatigue - Evening	
Fatigue - Morning	
Fibromyalgia	
Foggy Thinking	
Goiter	
Hair - Dry or Brittle	
Hair - Increased Facial or Body	
Hair - Scalp Loss	
Headaches	
Hearing Loss	
Heart Palpitations	
Hoarseness	
Hot Flashes	
Incontinence	
Infertility	
Irritable	
Libido Decreased	
Mania	

SYMPTOM CHECKLIST

- Memory Lapse
- Mood Swings
- Muscle Size Decreased
- Nails Breaking or Brittle
- Nervous
- Night Sweats
- Numbness - Feet or Hands
- OCD
- Panic Attacks
- PreMenstrual Dysphoric Disorder
- Pulse Rate Slow
- Rapid Aging
- Rapid Heartbeat
- Skin Thinning
- Sleep Disturbed
- Stamina Decreased
- Stress
- Sugar Cravings
- Sweating Decreased
- Swelling or Puffy Eyes/Face
- Tearful
- Triglycerides Elevated
- Urinary Urge Increased
- Uterine Fibroids
- Vaginal Dryness
- Water Retention
- Weight Gain - Hips
- Weight Gain - Waist



Lab Comments

CORTISOL (4x diurnal immunoassay) is normal in the morning but drops to low levels the remainder of the day indicating low adrenal reserve and HPA axis dysfunction. This usually is caused by stressors, a cortisol precursor deficiency (pregnenolone and progesterone), and/or nutritional deficiencies (low vitamins C and B5, low protein diet). The most common stressors that can cause adrenal exhaustion include: psychological stress (emotional), sleep deprivation, physical insults (surgery, injury, diseases), chemical exposure (environmental pollutants, excessive medications), and pathogenic infections (bacterial, viral, fungal). Depletion of cortisol by a chronic stressor often leads to symptoms such as fatigue, allergies (immune dysfunction), chemical sensitivity, cold body temp, and sugar craving. Because cortisol is important for the actions of thyroid hormone (T3) at the cellular/receptor/gene level it is important that cortisol is within physiological range before attempting thyroid therapy. Thyroid medication increases hepatic clearance of cortisol, which can lead to a further lowering of an already low cortisol level. Individuals taking thyroid therapy with weak adrenals often complain of being "tired but wired", particularly in the evening before bed. Adrenal support is worthwhile considering. Adequate sleep, gentle exercise, naps, meditation, proper diet (adequate protein), natural progesterone, adrenal extracts, herbs such as licorice, and nutritional supplements (vitamins C and B5) are some of the natural ways to help support adrenal function (consult with a health care provider for proper dosing). For additional information about strategies for supporting adrenal health and reducing stress(ors), the following books are worth reading: "Adrenal Fatigue", by James L. Wilson, N.D., D.C., Ph.D.; "The Cortisol Connection", by Shawn Talbott, Ph.D.; "The End of Stress As We Know It" by Bruce McEwen; "Awakening Athena" by Kenna Stephenson, MD.